

NOVINC, RYAN
13 MUSSON CL, FLOREY. 2615
Phone: 0432848508
Birthdate: 08/10/1997 Sex: M Medicare Number: 2816628782
Your Reference: 00287638 Lab Reference: 26-16891519-FBE-0
Laboratory: Laverty Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: HAEMATOLOGY (FBE-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
19:01

Clinical notes: Undergoing body composition change program.

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<u>HAEMATOLOGY</u>			
Date Collected	18 Sep 26		
Time Collected	08:40		
Specimen Type:	EDTA		
Hb	159 g/L	(130-180)	WBC 5.9 x10 ⁹ /L (4.0-11.0)
RCC	5.6 x10 ¹² /L	(4.5-6.5)	Neut 2.5 x10 ⁹ /L (2.0-7.5)
Hct	0.46	(0.40-0.54)	Lymp 2.5 x10 ⁹ /L (1.0-4.0)
MCV	83 fL	(79-99)	Mono 0.4 x10 ⁹ /L (0.2-1.0)
MCH	28 pg	(27-34)	Eos 0.4 x10 ⁹ /L (< 0.7)
MCHC	343 g/L	(320-360)	Baso 0.1 x10 ⁹ /L (< 0.2)
RDW	11.9 %	(10.0-17.0)	
Plat	192 x10 ⁹ /L	(150-400)	

HAEMATOLOGY: FBC parameters are within reference range.

Requested Tests : VBF*, TFT*, SCP*, GLU*, MBA*, LIP*, IFI*, FE*, FBE, DVI*,
AND*

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Your Reference: 00287638 Lab Reference: 26-16891519-TFT-0
Laboratory: Lavery Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: THYROID FUNCTION TEST (TFT-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
19:15

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Specimen Type: Serum
TSH 1.7 mIU/L (0.5-4.0)

Result(s) consistent with euthyroidism.

Please note that without a specific indication, Medicare does not fund FT4 and FT3 testing in patients with normal TSH results. If these tests are clinically indicated please contact the laboratory.

Requested Tests : VBF*, TFT, SCP*, GLU, MBA*, LIP*, IFI*, FE*, FBE, DVI*, AND*

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Your Reference: 00287638 Lab Reference: 26-16891519-GLU-0
Laboratory: Laverty Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: GLUCOSE (GLU-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
19:15

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	<u>SERUM/PLASMA GLUCOSE</u>
Fasting status	Fasting
Serum	5.1 mmol/L (3.4-5.4)

Normal glucose concentration.

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 Your Reference: 00287638 Lab Reference: 26-16891519-VBF-0
 Laboratory: Laverty Pathology
 Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: B12, FOLATE, R.C.FOLATE (VBF-0)
 Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
 19:19

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VITAMIN B12 AND FOLATE STUDIES

Vitamin B12 424 pmol/L (150-750)
 Serum Folate 33.7 nmol/L (> 9.0)

Serum Vitamin B12 Assay:

DEFICIENCY	BORDERLINE	SUFFICIENCY
<150 pmol/L	150 - 350 pmol/L	351 - 750 pmol/L

Vitamin B12 within reference interval. Tissue deficiency is unlikely.

Folate Interpretation:

DEFICIENCY	BORDERLINE	SUFFICIENCY
<4.5 nmol/L	4.5 - 9.0 nmol/L	>9.0 nmol/L

Serum Folate Assay:

In the absence of recent oral intake, a serum folate >9.0 nmol/L effectively rules out folate deficiency.

Red cell folates (RCF) are no longer processed routinely. If you have requested a RCF, and require a result for appropriate clinical reasons, this will need to be discussed and agreed with a Consultant Haematologist on +61290027085 or Dr. Lucinda Wallman, Consultant Pathologist in Immunology and Medical Director on telephone number +61 290057179

Requested Tests : VBF, TFT, SCP*, GLU, MBA*, LIP*, IFI*, FE*, FBE, DVI*, AND*

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Your Reference: 00287638 Lab Reference: 26-16891519-SCP-0
Laboratory: Laverty Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: ULTRA SENSITIVE CRP (SCP-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
19:46

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SERUM HIGH SENSITIVITY C-REACTIVE PROTEIN (CRP)
Request Number 16891519
Date Collected 18 Sep 26
Time Collected 08:40
hsCRP (< 4.00) mg/L 0.23

The CDC / AHA recommend the following hsCRP cut-off points (tertiles) for cardiovascular disease risk assessment:

hsCRP level (mg/L)	Relative Risk
<1.0	Low
1.0 - 3.0	Average
>3.0	High

The average of two CRP tests, ideally taken two weeks apart, produces a more stable estimate of this marker.

A CRP greater than 9.5mg/L should prompt a search for a source of infection or inflammation.

PLEASE NOTE: From 12/03/2026, High Sensitive CRP (hs-CRP) testing will be performed on the Siemens Atellica platform.

Requested Tests : VBF, TFT, SCP, GLU, MBA, LIP*, IFI*, FE, FBE, DVI*, AND

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Your Reference: 00287638 Lab Reference: 26-16891519-DVI-0
Laboratory: Lavery Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: VITAMIN D (DVI-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
20:11

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VITAMIN D

Haemolysis	Nil
Serum 25(OH) Vitamin D	92 nmol/L

Suggested decision limits for Vitamin D status:

Sufficiency	51 -200	nmol/L
Mild deficiency	25 - 50	nmol/L
Marked deficiency	< 25	nmol/L
Toxicity	>250	nmol/L

References: Vitamin D and health in adults in Australia and New Zealand:
Position Statement. MJA 2012 June 18; 196(11), 686-687.

Requested Tests : VBF, TFT, SCP, GLU, MBA, LIP*, IFI*, FE, FBE, DVI, AND

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Your Reference: 00287638 Lab Reference: 26-16891519-FE-0
Laboratory: Laverty Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: IRON STUDIES (FE-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
19:30

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	<u>IRON STUDIES</u>		
Specimen Type: Serum			
Serum Iron	39	umol/L	(10-30)
Transferrin	31	umol/L	(27-46)
Transferrin Saturation	64	%	(13-45)
Serum Ferritin	77	ug/L	(30-300)

Raised transferrin saturation may be the earliest sign of iron overload.
Suggest repeat fasting iron studies to confirm the finding.

Requested Tests : VBF, TFT, SCP*, GLU, MBA*, LIP*, IFI*, FE, FBE, DVI*, AND*

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 Your Reference: 00287638 Lab Reference: 26-16891519-MBA-0
 Laboratory: Laverty Pathology
 Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: SERUM CHEMISTRY (MBA-0)
 Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
 19:35

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<u>SERUM CHEMISTRY</u>			
Specimen Type: Serum			
Haemolysis	Nil		
Icterus	1+		
Lipaemia	Nil		
Sodium	140	mmol/L	(135-145)
Potassium	4.4	mmol/L	(3.5-5.2)
Chloride	103	mmol/L	(95-110)
Bicarbonate	26	mmol/L	(22-32)
Anion Gap	15	mmol/L	(7-17)
Urea	6.8	mmol/L	(3.0-8.5)
Creatinine	97	umol/L	(60-110)
eGFR	> 90		mL/min/1.73sqM
Urate	0.33	mmol/L	(0.20-0.42)
Bilirubin	22	umol/L	(< 24)
AST	34	U/L	(< 36)
ALT	59	U/L	(< 41)
GGT	20	U/L	(< 51)
Alkaline Phosphatase	57	U/L	(40-120)
Protein	75	g/L	(60-80)
Albumin	47	g/L	(38-50)
Globulin	28	g/L	(20-32)
Calcium	2.47	mmol/L	(2.10-2.60)
Corrected Calcium	2.39	mmol/L	(2.10-2.60)
Phosphate	0.76	mmol/L	(0.75-1.50)
Creatine Kinase	75	U/L	(< 294)

eGFR $\geq$ 90 mL/min/1.73m² usually indicates normal kidney function but does not exclude patients with early kidney damage (those with albuminuria, haematuria or abnormal kidney imaging).

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Laboratory: Lavery Pathology
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Name of Test: ANDROGENS (AND-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
19:36

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SERUM ANDROGENS			
Total Testosterone (Siemens)	30.2	nmol/L	(8.3-29)
Sex Hormone Binding Globulin	50	nmol/L	(11-71)
Calculated Free Testosterone	516.3	pmol/L	(255.0-725.0)

Requested Tests : VBF, TFT, SCP*, GLU, MBA, LIP*, IFI*, FE, FBE, DVI*, AND

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Laboratory: Laverty Pathology
Addressee: DR STEPHEN TANG Referred by: DR STEPHEN TANG

Name of Test: LIPID STUDIES (LIP-0)
Requested: 11/09/2026 Collected: 18/09/2026 Reported: 18/09/2026
20:17

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LIPID STUDIES

Specimen Type: Serum

Reference intervals are included for reference only, and interpretation / treatment goals should be guided by patient-specific cardiovascular risk assessment (see Australian Cardiovascular Risk Charts. Alternatively, the web-site www.cvdcheck.org.au can be accessed in order to complete a risk assessment for individual patients.)

Haemolysis	Nil
Icterus	1+
Lipaemia	Nil
Fasting status	Fasting
Total Cholesterol	6.3 mmol/L (3.0-5.2)
Triglycerides	1.3 mmol/L (0.5-1.7)
HDL Cholesterol	1.2 mmol/L (1.0-2.0)
LDL Cholesterol	4.5 mmol/L (1.5-3.4)
Non-HDL Cholesterol	5.1 mmol/L (< 3.4)
Cholesterol/HDL-C Ratio	5.2 (< 5.0)

NVDPA TARGET LIPID RANGES (MMOL/L) FOR PATIENTS AT HIGH / MODERATE RISK OF CARDIOVASCULAR DISEASE:

TOTAL CHOLESTEROL	<4.0
TRIGS (FASTING)	<2.0
HDL-C	>= 1.0
LDL-C	<2.0
NON HDL-C	<2.5

Increased LDL-C can be seen in hypothyroidism and nephrotic syndrome. Primary causes include polygenic hypercholesterolaemia, Familial Hypercholesterolaemia, Familial Defective ApoB-100 and Familial Combined Hyperlipoproteinaemia. Increased LDL-C is a risk factor for CVD. Assess overall risk and consider treatment.

Lipoprotein X may be seen in patients with cholestatic liver disease and lecithin-cholesterol acyl transferase deficiency.

Requested Tests : VBF, TFT, SCP, GLU, MBA, LIP, IFI*, FE, FBE, DVI, AND

